The emergency summary that travels with the patient

MediChain gives every patient a health ID they carry as a QR code or NFC card. A responder reads the essentials in seconds; the care team works from one record; and the patient sees every access to it.

Why now

Missing records delay care: in a survey of 40 Limpopo hospitals, patients waited longer and some were "treated without medical history" [1]. Courts have found that "numerous South African public health facilities fail to meet their record-keeping obligations" [2]. The national digital-health strategy asks for a person-centred record and a common patient identifier [3].

What is different

- **Seconds, not phone calls:** QR, NFC or card number, on an approved device.
- **Visible to the patient:** every read, emergency included, in their access history.
- **Provable:** access records are signed and stored write-once.
- **Patient-held consent:** access is granted and withdrawn by the patient.

Traction

Live
on AWS in Cape Town since October 2026

7
roles on one record, 116 screens

1,045
server tests, all passing

2nd
place, Rust Africa Hackathon 2026

Business

A per-facility subscription for private clinics, hospital groups and EMS operators; free for patients. One server in Cape Town runs the whole product today for under USD 60 a month.

The ask

One clinic, hospital or EMS partner for a supervised pilot of the emergency summary and handover, and introductions to health-technology funders.

Lukau Invasion (Pty) Ltd · 2026/600804/07 · Keorapetswe Kgoatlha, founder-developer · kkgawatlh9@gmail.com

Sources

1. Marutha, N. S. & Ngoepe, M. (2017). The role of medical records in the provision of public healthcare services in the Limpopo province of South Africa. *South African Journal of Information Management*, 19(1), a873. https://doi.org/10.4102/sajim.v19i1.873
2. Kolawole, O. (2024). Non-compliant health record-keeping in South Africa: Judicial responses, and progress towards a digital remedy. *The African Journal of Information and Communication*, (33), 1–19. https://doi.org/10.23962/ajic.i33.18892
3. National Department of Health (2019). *National Digital Health Strategy for South Africa 2019–2024*. https://knowledgehub.health.gov.za/system/files/elibdownloads/2023-04/national-digital-strategy-for-south-africa-2019-2024-b.pdf

One evening, two endings

A fictional but ordinary case: Thabo, 55, has type 2 diabetes and high blood pressure and is allergic to aspirin. At 18:40 he collapses at a taxi rank in Tembisa.

Time	Today, with paper folders	With MediChain
18:52 ambulance arrives	Thabo cannot speak. No one on scene knows his conditions, medicines or allergies.	The paramedic scans the QR on Thabo's phone lock screen, or taps his card: blood type, allergies, medicines and next of kin appear.
18:58 treatment starts	Standard chest-pain protocols often include aspirin, so a responder must stop and ask. Nobody can answer.	The aspirin allergy is the first thing on screen. The grant lasts 15 minutes and covers Thabo only.
19:30 hospital handover	His folder is at his clinic, 20 km away. Staff phone family members.	The receiving doctor opens the same record: notes, recent results and current medicines.
Next morning	Thabo has no way to know who read his information.	Thabo's access history lists the paramedic and the doctor, with times and reasons.

This is an illustrative scenario, not a measured outcome. MediChain makes no claim about mortality or error rates; those will be measured in a pilot.

The evidence behind the problem

- **Limpopo, 40 public hospitals:** missing records "negatively affected timely and effective healthcare service delivery"; staff were "unable to assist patients or treat them immediately" [1].
- **National case law, 2024:** poor record-keeping undermines patients' own claims, because facilities fail their legal obligations [2].
- **Khayelitsha Hospital emergency centre:** months of folders stayed un-scanned and unavailable electronically [3] (one hospital, 2012–2013).
- **Policy:** the National Digital Health Strategy calls for a complete electronic record and a common identifier [4].

Who gains what

Who	Before	After
Patients	History scattered across folders; no view of who reads it	Carry their own summary; approve and withdraw access; read their notes, results and messages
Paramedics	Treat blind or wait for a phone call	Allergies and medicines on scene, on an approved device
Doctors and nurses	Re-take histories; handovers on paper	One shared record; notifications only about their own patients
Pharmacists and laboratories	Prescriptions and requests that cannot be traced	Prescriptions and specimens tied to the patient and the prescriber
Facilities	Disputes argued from missing paper	A signed, write-once record of every access

Built for South African reality

- **Load-shedding and weak signal:** the QR carries a printed summary that still shows offline, marked unverified.
- **Any phone:** web apps that install from the browser, with no app-store download or data-heavy update.
- **Local hosting:** data stays in South Africa (AWS Cape Town).
- **Six languages prepared:** English, isiZulu, Kiswahili, Hausa, Amharic and French interface bundles exist; English is complete.

How a pilot will prove it

Measure	How
Seconds to critical information	From first contact to allergies on screen, against the current route
Handover completeness	Share of handovers with allergies, medicines and contacts recorded
Patient understanding	Can patients say who opened their record, and why?
Adoption	Active staff per shift; workflows finished without paper

Sources

1. Marutha, N. S. & Ngoepe, M. (2017). The role of medical records in the provision of public healthcare services in the Limpopo province of South Africa. *South African Journal of Information Management*, 19(1), a873. https://doi.org/10.4102/sajim.v19i1.873
2. Kolawole, O. (2024). Non-compliant health record-keeping in South Africa: Judicial responses, and progress towards a digital remedy. *The African Journal of Information and Communication*, (33), 1–19. https://doi.org/10.23962/ajic.i33.18892
3. Ohuabunwa, E. U., Sun, J., Jubanyik, K. J. & Wallis, L. A. (2016). Electronic medical records in low to middle income countries: the case of Khayelitsha Hospital, South Africa. *African Journal of Emergency Medicine*, 6(1), 38–43. https://doi.org/10.1016/j.afjem.2015.06.003
4. National Department of Health (2019). *National Digital Health Strategy for South Africa 2019–2024*. https://knowledgehub.health.gov.za/system/files/elibdownloads/2023-04/national-digital-strategy-for-south-africa-2019-2024-b.pdf